

REVENUE CYCLE
KNOWLEDGE BANK

Healthcare Insurance and RCM Terminology Guide

100+ essential terms in practical, plain language

Designed for

RCM learners, patient access teams, billers, payment posters, AR analysts, denial specialists, team leads, and managers.

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How to Use This Guide

Plain-language learning resource

Definitions are paraphrased for education. A term can have a more specific meaning in a health plan, payer policy, provider contract, law, code set, or operating procedure. Always verify the controlling source.

For learners

Start with Coverage & plan basics, Cost sharing, and Claims & transactions. Then move to adjudication, denials, and financial operations.

For operations teams

Use the “Why it matters” column to connect terminology to claim outcomes and day-to-day work.

For trainers

Use terms as flashcards, quiz prompts, scenario vocabulary, and sandbox tooltips.

For leaders

Standardize local definitions before using terms in KPIs, SOPs, audits, and performance targets.

Five Frequently Confused Pairs

- **Rejection vs denial:** A rejection occurs before full adjudication; a denial is an adjudicated nonpayment or reduction.
- **EFT vs ERA:** EFT transfers money; ERA explains claim payment and adjustments.
- **Referral vs authorization:** A referral directs care; authorization is payer approval when required. A plan may require one, both, or neither.
- **Corrected claim vs appeal:** A corrected claim changes supported claim data; an appeal disputes a processed decision.
- **Charge vs allowed amount:** The charge is the provider’s submitted price; the allowed amount is the plan-recognized amount for covered services.

Coverage & plan basics

Term	Abbrev.	Plain-language meaning	Why it matters in RCM / example
Allowed amount	—	Maximum amount the plan recognizes for a covered service.	A \$500 charge may have a \$320 allowed amount; payment and patient share are generally calculated from \$320.
Benefit year	—	Period used to administer benefits, often 12 months.	Deductible and out-of-pocket accumulators may reset when the benefit year restarts.
Covered service	—	Service eligible for benefits when plan conditions are met.	Coverage can still depend on network, authorization, medical necessity, or frequency rules.
Excluded service	—	Service the plan specifically does not cover.	Verify the plan document and required patient notice before transferring liability.
Health coverage	—	Arrangement that pays or reimburses eligible healthcare costs.	Coverage may come from commercial insurance, an employer plan, Medicare, Medicaid, or CHIP.
Premium	—	Regular amount paid to keep coverage active.	Premiums are separate from claim-level deductible, copay, and coinsurance.
Policyholder	—	Person or entity that holds the insurance contract.	The patient may be a dependent rather than the policyholder.
Subscriber	—	Person enrolled as the primary member under a plan.	Subscriber name, ID, and relationship must match payer records.
Dependent	—	Person covered under another enrollee’s plan.	Dependent eligibility and relationship errors can cause denials.
Member ID	—	Identifier assigned by the payer to the enrolled person.	An incorrect member ID can result in rejection, not-on-file status, or wrong-payer billing.
Group number	—	Identifier for an employer or plan group.	Used with member details to locate the correct benefit package.
Plan document	—	Contract or evidence of coverage describing benefits and rules.	When a glossary differs from the plan, the applicable plan terms generally govern.
Summary of Benefits and Coverage	SBC	Standard summary of a health plan’s benefits and cost sharing.	Useful for basic benefit review, but detailed claim questions may require the full plan document.

Cost sharing

Term	Abbrev.	Plain-language meaning	Why it matters in RCM / example
Deductible	—	Amount a member pays for covered services before the plan begins paying as specified.	A service may apply fully or partly to the remaining deductible.
Copayment	Copay	Fixed member amount for a covered service.	A plan may assign different copays to primary care, specialist, urgent care, and emergency services.
Coinsurance	—	Member percentage of the allowed amount, usually after deductible conditions are met.	With 20% coinsurance on a \$300 allowed amount, the member share is \$60.
Cost sharing	—	Member-paid amounts such as deductible, copay, and coinsurance.	Cost-sharing rules must be read with network and benefit provisions.
Out-of-pocket maximum	OOP max	Plan-year limit on specified member cost sharing for covered services.	Not every amount necessarily counts; verify network and benefit rules.
Patient responsibility	PR	Amount assigned to the patient after valid benefit adjudication.	Confirm secondary coverage, notices, contract terms, and applicable rules before billing.
Balance billing	—	Billing a patient for the difference between a provider charge and the plan’s allowed amount.	May be restricted by contracts and law; do not treat every unpaid balance as billable.
Self-pay	—	Patient pays without billing an insurer for the service.	Use the approved self-pay workflow, estimate, discount, and financial-assistance policy.
Good Faith Estimate	GFE	Advance estimate of expected charges in situations covered by applicable requirements.	Supports price transparency for uninsured or self-pay patients.

Network & plan models

Term	Abbrev.	Plain-language meaning	Why it matters in RCM / example
In-network provider	—	Provider contracted with the plan under negotiated terms.	Usually associated with lower member cost sharing and contractual write-offs.
Out-of-network provider	OON	Provider without the applicable network contract.	Benefits, authorization, reimbursement, and patient liability may differ.
Preferred provider	—	Provider the plan encourages members to use, commonly through network participation.	Confirm product-specific participation, not only payer-level contracting.
Health Maintenance Organization	HMO	Plan model commonly using a defined network and coordinated care rules.	Referral and out-of-network rules may be stricter, subject to the plan.
Preferred Provider Organization	PPO	Plan model generally allowing broader provider choice with different network cost sharing.	Out-of-network benefits may exist but can create higher member costs.
Exclusive Provider Organization	EPO	Plan model generally limiting routine coverage to a network, except as specified.	Verify network status carefully before nonemergency services.
Point of Service plan	POS plan	Plan that combines network coordination with some out-of-network options.	Referrals and cost sharing may vary based on how care is accessed.
High-Deductible Health Plan	HDHP	Plan meeting applicable rules for higher deductibles and related cost-sharing structure.	Do not assume HSA eligibility without checking current requirements.
Capitation	—	Payment arrangement providing a set amount for defined members or services over a period.	A claim may be reported for encounter purposes even when paid under capitation.
Fee for service	FFS	Payment approach based on separately billed eligible services.	Claims are adjudicated using coding, coverage, and payment rules.

Government programs

Term	Abbrev.	Plain-language meaning	Why it matters in RCM / example
Medicare	—	Federal health coverage program for eligible individuals.	Includes multiple parts and plan arrangements with different billing requirements.
Medicaid	—	Federal-state program providing coverage to eligible populations.	Benefits, payer structure, and billing rules vary by state and program.
Children’s Health Insurance Program	CHIP	Coverage program for eligible children in families meeting program requirements.	Eligibility and administration vary by state.
Medicare Administrative Contractor	MAC	Contractor that processes specified Medicare Fee-for-Service work by jurisdiction.	Provider enrollment, claims, remittance, and appeals may route through the applicable MAC.
Medicare Advantage	MA	Medicare benefits delivered through a private plan approved to administer Medicare coverage.	Follow the MA plan’s network, authorization, claims, and appeal requirements.
Medicare Secondary Payer	MSP	Rules determining when Medicare pays after another responsible payer.	Collect accurate accident, employment, and other coverage information.

Patient access

Term	Abbrev.	Plain-language meaning	Why it matters in RCM / example
Eligibility verification	—	Process of confirming active coverage and benefit information.	Verify coverage for the date of service and retain the response.
Benefits verification	—	Review of coverage details for a planned or billed service.	Check deductible, copay, coinsurance, limits, exclusions, and network rules.
Prior authorization	PA	Payer approval required before specified services under plan rules.	Approval may need to match code, units, dates, provider, and location.
Precertification	—	Advance payer review or approval process, often used for facilities or services.	Terminology and requirements vary by payer.
Referral	—	Direction from one provider to another when required by the plan or care model.	A referral is not always the same as authorization.
Medical necessity	—	Payer or program criteria used to assess whether a service is clinically appropriate for coverage.	Use current policy and supporting documentation for appeals.
Benefit limit	—	Restriction on visits, days, units, dollars, or other covered utilization.	Check accumulators before service and during denial review.
Coordination of benefits	COB	Method for determining payment order when more than one plan may cover a person.	Bill primary, then pass prior-payer information to secondary as required.

Claims & transactions

Term	Abbrev.	Plain-language meaning	Why it matters in RCM / example
Claim	—	Request for payment or benefit consideration for healthcare items or services.	A claim must contain accurate patient, provider, service, diagnosis, and payer data.
Professional claim	837P / CMS-1500	Claim format commonly used for professional services.	Electronic claims generally use the 837P transaction; paper billing may use CMS-1500.
Institutional claim	837I / UB-04	Claim format commonly used for facility services.	Electronic claims generally use the 837I transaction; paper billing may use UB-04.
Dental claim	837D	Electronic claim transaction for dental services.	Dental payer rules and coding differ from medical claims.
Electronic Data Interchange	EDI	Structured electronic exchange of healthcare transactions.	Supports claims, eligibility, status, remittance, and related workflows.
Clearinghouse	—	Entity that edits, translates, and routes electronic transactions.	A clearinghouse acceptance does not always mean the payer accepted the claim.
Trading partner	—	Organization exchanging EDI transactions under established arrangements.	Payers, providers, clearinghouses, and vendors can be trading partners.
999 acknowledgment	999	EDI acknowledgment indicating whether a transaction set passed structural validation.	It does not confirm final claim adjudication.
Claim acknowledgment	277CA	Transaction reporting claim acceptance or rejection at an early processing stage.	Use it to correct front-end issues before timely-filing risk grows.
Claim status inquiry and response	276/277	Electronic transactions used to request and return claim-status information.	Useful for automated follow-up after claim acceptance.
Eligibility inquiry and response	270/271	Electronic transactions used to request and return eligibility and benefit information.	Responses are point-in-time and do not guarantee payment.
Electronic remittance advice	835 / ERA	Electronic transaction containing claim payment and adjustment information.	Supports automated posting and standardized adjustment reporting.
Electronic funds transfer	EFT	Electronic movement of payment funds.	EFT moves money; ERA explains how claims were adjudicated.
Payer ID	—	Identifier used to route electronic transactions to a payer or processor.	The wrong payer ID can misroute or reject a claim.
National Provider Identifier	NPI	Standard identifier for covered healthcare providers.	Billing, rendering, referring, and facility NPIs must populate the proper fields.
Taxonomy code	—	Code describing provider type, classification, or specialization.	Mismatch with payer enrollment can cause rejection or denial.
Tax Identification Number	TIN	Tax identifier associated with the billing entity.	Must align with payer enrollment and contracting records.

Coding & claim data

Term	Abbrev.	Plain-language meaning	Why it matters in RCM / example
International Classification of Diseases, Tenth Revision, Clinical Modification	ICD-10-CM	Diagnosis classification used to report conditions and reasons for care.	Code selection and sequencing must be supported by documentation and current rules.
Current Procedural Terminology	CPT	Proprietary code set used to report many professional services and procedures.	Use only authorized current resources; CPT is owned by the American Medical Association.
Healthcare Common Procedure Coding System	HCPCS	Coding system that includes products, supplies, services, and procedures.	Level II commonly covers items and services not represented in CPT.
Modifier	—	Code element providing additional information about a reported service.	Never add a modifier solely to obtain payment; documentation and rules must support it.
Place of service	POS	Code indicating where a professional service occurred.	POS can affect coverage, payment, and facility versus nonfacility rates.
Type of bill	TOB	Institutional claim code describing facility type and bill frequency.	Incorrect TOB can misroute or reject an institutional claim.
Revenue code	—	Institutional claim code identifying the department or service category.	Must align appropriately with service details and payer edits.
Diagnosis pointer	—	Claim element linking a service line to relevant diagnosis codes.	Incorrect linkage may trigger medical-necessity edits.
Units	—	Quantity of a billed item or service.	Verify units against documentation, policy, and authorization.

Adjudication & remittance

Term	Abbrev.	Plain-language meaning	Why it matters in RCM / example
Adjudication	—	Payer process for evaluating a claim and determining payment, adjustment, or denial.	Review line-level outcomes, not just the claim header.
Explanation of Benefits	EOB	Member-facing explanation of how a claim was processed.	An EOB is not usually a provider invoice.
Remittance advice	RA	Provider-facing explanation of claim adjudication and payment.	May be electronic as an ERA or provided as paper remittance.
Claim Adjustment Group Code	—	Code indicating the general category of financial responsibility.	Common groups include contractual obligation, patient responsibility, other adjustment, and payer-initiated reduction.
Claim Adjustment Reason Code	CARC	Standard code explaining why payment differs from the submitted amount.	Read with the group code and any associated RARC.
Remittance Advice Remark Code	RARC	Code adding detail about an adjustment or remittance processing.	Often clarifies missing information, policy context, or an alert.
Contractual obligation	CO	Group code generally assigning an adjustment to provider contractual responsibility.	Do not transfer CO amounts to the patient unless permitted through a separate valid basis.
Other adjustment	OA	Group code used for adjustments not assigned as standard provider or patient responsibility.	Review the CARC, RARC, and payer explanation.
Payer-initiated reduction	PI	Group code indicating a payer-initiated adjustment in specified situations.	Determine responsibility from the complete remit and applicable rules.
Provider-level balance adjustment	PLB	ERA adjustment not tied to one specific claim line.	May represent recoupment, interest, incentive, or another provider-level transaction.
Contractual adjustment	—	Difference written off under an applicable provider-payer agreement.	Validate the allowed amount and contract before posting.
Takeback / recoupment	—	Recovery of a prior payer payment.	Trace the original claim, reason, remit, and appeal rights.
Underpayment	—	Payment below the amount expected under applicable terms.	Compare actual reimbursement with a reliable contract model.
Overpayment	—	Payment exceeding the amount properly due.	Route through approved refund or recoupment workflow.

Denials & follow-up

Term	Abbrev.	Plain-language meaning	Why it matters in RCM / example
Rejection	—	Claim not accepted into adjudication because of front-end or transaction issues.	Correct and resubmit promptly; this differs from a processed denial.
Denial	—	Adjudicated decision not to pay all or part of a claim or service.	Identify payer reason, root cause, correction or appeal route, and filing limit.
Corrected claim	—	Resubmission changing supported claim information using the payer-required indicator.	Use for claim data corrections, not to bypass a valid denial without changes.
Reopening	—	Request to revise a prior determination under a payer or program process.	Availability and rules vary by payer and issue.
Reconsideration	—	Request for another review of a claim or payment decision.	Follow the applicable appeal level, deadline, and evidence rules.
Appeal	—	Formal request to review an adverse benefit or payment decision.	State the disputed decision, evidence, supporting basis, and requested remedy.
Timely filing limit	TFL	Deadline for initial claim, corrected claim, or appeal submission.	Keep acceptance evidence and distinguish each type of deadline.
Proof of timely filing	POTF	Evidence a claim or appeal was submitted or accepted within the deadline.	Examples include clearinghouse acceptance, payer receipt, or approved tracking evidence.
Root cause analysis	RCA	Structured method for finding the underlying source of a problem.	Classify denials by true upstream cause rather than only payer code.
A/R aging	—	Grouping outstanding receivables based on age.	Common buckets are 0-30, 31-60, 61-90, 91-120, and over 120 days.
Work queue	—	System list routing accounts or tasks for action.	Queue logic should support priority, ownership, deadlines, and disposition.

Financial & operational

Term	Abbrev.	Plain-language meaning	Why it matters in RCM / example
Accounts receivable	A/R	Amounts owed to a healthcare organization for services provided.	Monitor total balance, aging, payer mix, credits, and collectability.
Days in A/R	—	Estimated number of revenue days represented by receivables.	Use a governed gross or net formula consistently.
Discharged not final billed	DNFB	Discharged encounters not ready for final billing.	Often affected by coding, documentation, charge, or discharge-processing delays.
Clean claim rate	—	Share of claims accepted without defined correction or front-end failure.	Document the organization’s exact clean-claim definition.
First-pass resolution rate	FPR	Share of claims resolved on first adjudication without rework.	Define what counts as resolved and the population used.
Denial rate	—	Share of adjudicated claims or dollars initially denied.	Label whether the metric uses claim count, lines, charges, or expected dollars.
Net collection rate	NCR	Collections compared with the defined collectible amount.	Use matched periods and documented exclusions.
Gross collection rate	GCR	Collections compared with gross charges.	Useful for trend context but heavily affected by charge levels and payer mix.
Cost to collect	—	Revenue-cycle operating cost divided by defined patient-service cash collections.	Document included labor, vendor, technology, and shared costs.
Credit balance	—	Negative account balance indicating excess payment or adjustment.	Investigate refunds, recoupments, posting errors, and unapplied cash.
Write-off	—	Approved removal of an amount from receivables under policy.	Track contractual, administrative, bad debt, timely filing, and other categories separately.
Bad debt	—	Patient balance deemed uncollectible under approved policy and accounting treatment.	Distinguish bad debt from charity care, contractual adjustment, and billing error.

Knowledge Check

1. Which transaction explains electronic claim payment and adjustments?

A. 270/271 B. 835/ERA C. 276/277 D. 999

2. What is the difference between a rejection and a denial?

Write a one-sentence explanation.

3. If a service has a \$300 allowed amount and 20% coinsurance after deductible, what is the member coinsurance?

Answer: _____

4. Which should generally be used to change supported claim data: corrected claim or appeal?

Answer: _____

5. Name the three code elements commonly reviewed together on a remittance adjustment.

Answer: _____

6. Why does eligibility verification not guarantee payment?

Answer: _____

7. What is the difference between EFT and ERA?

Answer: _____

8. Which metric estimates how many days of revenue are tied up in receivables?

Answer: _____

Suggested answers

1: 835/ERA. 2: A rejection occurs before full adjudication; a denial reflects an adjudicated decision. 3: \$60. 4: Corrected claim. 5: Group code, CARC, and RARC. 6: Coverage and benefits are point-in-time information and payment also depends on claim, coding, authorization, policy, and other rules. 7: EFT transfers funds; ERA explains adjudication. 8: Days in A/R.

Sources, Licensing Note, and Disclaimer

This guide uses original, plain-language educational explanations informed by official public references. The guide does not reproduce a complete official glossary or proprietary code set.

Official reference links

- **CMS health insurance terms:** <https://www.cms.gov/initiatives/your-patient-rights/medical-bill-rights/get-help/medical-bill-guides-resources/health-insurance-terms-you-should-know>
- **HealthCare.gov glossary:** <https://www.healthcare.gov/glossary/>
- **CMS glossary and acronyms:** <https://www.cms.gov/glossary>
- **CMS payment and remittance advice:** <https://www.cms.gov/medicare/coding-billing/electronic-billing/health-care-payment-remittance-advice>
- **X12 Claim Adjustment Reason Codes:** <https://x12.org/codes/claim-adjustment-reason-codes>
- **X12 Remittance Advice Remark Codes:** <https://x12.org/codes/remittance-advice-remark-codes>

Coding and intellectual-property note

CPT is a proprietary code set of the American Medical Association. This guide defines the term at a high level and does not provide CPT code descriptions or reproduce proprietary code-set content. Users should access properly licensed, current coding resources.

Educational disclaimer

For education and workflow support only. This guide is not legal, clinical, coding, accounting, contractual, compliance, benefits, or payer-specific advice. Definitions, coverage requirements, billing rules, code sets, and plan terms change. The applicable plan document, payer policy, provider contract, official code set, law, and approved organizational procedure govern.

Content improvement note

For LMS use, each term can become a searchable glossary card with a definition, scenario, related terms, quiz question, and sandbox tooltip. Keep version dates and source links visible so learners know when content was last reviewed.